

Policy on Facility, Engineering Support and Utility Systems

«Hospital Name»

Document control

Document No.	HCO/FMS/POL/04	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Engineering In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

The organisation has a programme for the facility, engineering support services and utility system.

1. Purpose

This policy sets out how «Hospital Name» runs its facility, engineering support services and utility programme. It covers:

- Planning, inventorying, operating and maintaining utility and engineering equipment
- Keeping competent maintenance personnel available round the clock
- Guiding equipment replacement and disposal

This policy covers facility, engineering support and utility systems specifically. Related duties — like patient assessment, clinical care, medication management, patient rights, infection control, quality and safety monitoring, or hospital governance — are covered in the hospital's other policies, not repeated here.

Words marked like this are defaults. A blank marked _____ must be filled before issue.

2. Scope

This policy applies to «Hospital Name» staff involved in facilities, utilities, medical gases, and fire or non-fire emergencies, including:

- Engineering, biomedical, nursing and departmental leaders

- The Engineering In-Charge
- The Medical Superintendent
- Departmental leaders
- The Quality Coordinator

This policy covers facility, engineering support and utility systems specifically. Related duties are covered in the hospital's other policies, not repeated here, including:

- Patient assessment
- Clinical care
- Medication management
- Patient rights
- Infection control
- Quality and safety monitoring
- Hospital governance

3. Policy standards

«Hospital Name» plans utility and engineering equipment against services and the strategic plan. All equipment is inventoried with unique identifiers. Implemented operational and maintenance plans cover every system. Calibration is kept current. Competent personnel are available for every shift. Maintenance is contactable round the clock. Downtime on critical equipment is tracked from complaint to completion. Equipment replacement and disposal follows written guidance.

Staff follow written guidance and keep the records listed in the traceability table.

4. Non-negotiable rules

1. Do not run utility or engineering equipment that is not in the current equipment inventory with a unique identifier.
2. Do not run critical utility equipment (diesel generator, lifts, uninterruptible power supply, fire-related equipment, dialysis reverse-osmosis plant, water pumps) without an implemented, documented operational and maintenance plan.
3. Do not let utility equipment become overdue for calibration without a corrective action in place.
4. Do not operate a shift without a named competent person available for each plant system that is running.
5. Do not leave the maintenance escalation matrix unavailable at the nursing station and departments during any shift.
6. Do not condemn or dispose of utility or engineering equipment without following the written equipment-replacement and disposal guidance.
7. Do not bypass the stop-work authority in section 6 when the trigger conditions are met.
8. Staff who see a FMS.4 rule broken report it the same shift to the Engineering In-Charge or the Medical Superintendent.

5. What we do

5.1 The organisation plans for utility and engineering equipment in accordance with its services and strategic plan.

The organisation plans utility and engineering equipment to match services and the strategic plan, including future needs (diesel generator, chiller). Plans are implemented and reviewed at a defined interval. Selection, rental, update or upgrade is collaborative (end-user, management, finance, engineering).

The Engineering In-Charge holds the current plan.

5.2 Equipment is inventoried, and proper logs are maintained as required

Equipment is inventoried and logs are maintained. Each item has a unique identifier. Quality-conformance certificates, factory test certificates and installation reports are kept where they apply.

A plant item with no identity number does not satisfy this element.

5.3 The documented operational and maintenance (preventive and breakdown) plan is implemented.

The documented operational and maintenance (preventive and breakdown) plan is implemented for utility and engineering equipment, electrical systems, water, heating-ventilation-air-conditioning, facility and furniture — including transformers, low-tension and high-tension panels, lifts, tanks, reverse-osmosis and sewage-treatment if present, chillers, air-handling units and filters.

Running critical utility equipment with no implemented plan is a stop-work trigger (section 6).

5.4 Utility equipment, are periodically inspected and calibrated (wherever applicable) for their proper functioning.

Utility equipment is periodically inspected and calibrated where applicable (for example steam-steriliser pressure gauges, medication-refrigerator temperature gauges), in-house or outsourced, with traceability to prescribed standards.

The Engineering In-Charge holds the calibration schedule.

5.5 Competent personnel operate, inspect, test and maintain equipment and utility systems.

Competent personnel (qualification, experience or training) operate, inspect, test and maintain equipment and utility systems. Enough supervisors and tradespeople, including fire-safety and electrical-safety trained staff, plus tools and personal protective equipment, are available.

A plant running with no competent person named for that shift is escalated to the Engineering In-Charge.

5.6 Maintenance staff is contactable round the clock for emergency repairs

Maintenance staff are contactable round the clock for emergency repairs. An escalation matrix (who to call if the person on duty cannot complete the job) is at the nursing station and departments.

A night call with no answer path does not satisfy this element.

5.7 Downtime for critical equipment breakdowns is monitored from reporting to inspection and implementation of corrective actions.

Downtime for critical engineering and utility equipment breakdowns is monitored from reporting to inspection and corrective action. At a minimum the critical list includes diesel generator, lifts, uninterruptible power supply, fire-related equipment, dialysis reverse-osmosis and water pumps. A complaint register records receipt, job allotment and user-ratified completion.

Start of downtime is complaint time; end is user-ratified completion.

5.8 Written guidance supports equipment replacement, identification of unwanted material and disposal.

Written guidance supports equipment replacement, identification of unwanted material and disposal. Unusable utility and engineering equipment is condemned in a systematic way. Records are kept.

6. Stop-work authority

Do not run critical utility equipment (as this hospital names it: at a minimum diesel generator, lifts, uninterruptible power supply, fire-related utility, reverse-osmosis plant for dialysis, water pumps) when there is no implemented operational and maintenance plan, or when the equipment is known unsafe.

Stop-work applies to starting or continuing that equipment. Life-saving care continues under the hospital's emergency continuity plan.

The person who stops tells the Engineering In-Charge and the Medical Superintendent the same shift. Refusing unsafe utility equipment is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that FMS.4 is resourced and followed.
Engineering In-Charge	Owns day-to-day implementation and records for this standard.
Quality Coordinator	Audits this policy quarterly; holds training acknowledgements.
departmental leaders	Run the department-level duties this

	standard names.
--	-----------------

8. Quality monitoring (RCA → CAPA)

The Quality Coordinator audits this policy quarterly. The audit reviews:

- Records for a sample of this standard's objective elements, checked against the What-we-do steps.
- Documentary evidence is on file for each asterisked objective element in the sample.
- CORE objective elements show no critical gaps in the sample.
- Stop-work events (if any) are logged with outcome.

Root-cause analysis is required when a gap found in this audit remains open beyond 90 days.

This policy is reviewed annually, and sooner after a related facility change, utility failure, equipment recall or fire-plan change.

9. Training and staff acknowledgement

Staff covered by this policy are trained at induction and once a year after that. Training covers the What-we-do steps, non-negotiables and stop-work (if present).

Staff acknowledgement

I have read the Policy on Facility, Engineering Support and Utility Systems of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

10. Distribution

This policy is distributed to:

- Medical Superintendent
- Engineering In-Charge
- Quality Coordinator
- departmental leaders
- staff covered by FMS.4

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
CORE	Core objective element (NABH)
FMS	Facility Management and Safety (NABH Hospitals 6th Edition chapter)
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element
RO	Reverse Osmosis

12. Traceability to NABH HCO Full Accreditation 6th Edition FMS.4

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
FMS.4.a	Commitment	The organisation plans for utility and engineering equipment in accordance with its services and strategic plan.	Section 3; 5.1	Engineering In-Charge
FMS.4.b	Commitment	Equipment is inventoried, and proper logs are maintained as required.	Section 3; 5.2	Engineering In-Charge
FMS.4.c	CORE*	The documented operational and maintenance (preventive and breakdown) plan is implemented.	Section 3; 5.3; Section 6 Stop-work	Engineering In-Charge
FMS.4.d	Commitment	Utility equipment, are periodically inspected and calibrated (wherever applicable) for their proper functioning.	Section 3; 5.4	Engineering In-Charge

FMS.4.e	Commitment	Competent personnel operate, inspect, test and maintain equipment and utility systems.	Section 3; 5.5	Engineering In-Charge
FMS.4.f	Commitment	Maintenance staff is contactable round the clock for emergency repairs.	Section 3; 5.6	Engineering In-Charge
FMS.4.g	Achievement	Downtime for critical equipment breakdowns is monitored from reporting to inspection and implementation of corrective actions.	Section 3; 5.7	Engineering In-Charge
FMS.4.h	Commitment*	Written guidance supports equipment replacement, identification of unwanted material and disposal.	Section 3; 5.8	Engineering In-Charge

13. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

FMS.4.a — The organisation plans for utility and engineering equipment in accordance with its services and strategic plan.

- Written utility and engineering equipment plan matched to services and the strategic plan, including future needs (for example diesel generator, chiller).
- Periodic review record of the plan.
- Record showing collaborative selection (end-user, management, finance, engineering) for equipment decisions.

FMS.4.b — Equipment is inventoried, and proper logs are maintained as required.

- Equipment inventory with a unique identifier per item.

- Quality-conformance, factory test or installation certificate on file where applicable.
- Log-completeness check confirming no item without an identity.

FMS.4.c — The documented operational and maintenance (preventive and breakdown) plan is implemented.

- Documented operational and maintenance (preventive and breakdown) plan for utility and engineering equipment.
- Preventive-maintenance schedule and completion log for the critical-equipment list.
- Breakdown-response record showing the plan was actually followed.

FMS.4.d — Utility equipment, are periodically inspected and calibrated (wherever applicable) for their proper functioning.

- Calibration schedule for utility equipment (for example steam-steriliser pressure gauges, medication-refrigerator temperature gauges).
- Calibration certificate with traceability to a prescribed standard.
- In-house or outsourced calibration record.

FMS.4.e — Competent personnel operate, inspect, test and maintain equipment and utility systems.

- Competency or training record for staff operating utility equipment.
- Roster showing a named competent person for each shift.
- Escalation record for any shift with no competent person named.

FMS.4.f — Maintenance staff is contactable round the clock for emergency repairs.

- Escalation matrix displayed at the nursing station and departments.
- On-call roster for maintenance staff, round the clock.
- Night-call response-time log.

FMS.4.g — Downtime for critical equipment breakdowns is monitored from reporting to inspection and implementation of corrective actions.

- Complaint register logging receipt, job allotment and user-ratified completion for critical equipment (diesel generator, lifts, UPS, fire-related, dialysis RO, water pumps).
- Downtime-duration record from complaint time to ratified completion.
- Trend report reviewed against the critical-equipment list.

FMS.4.h — Written guidance supports equipment replacement, identification of unwanted material and disposal.

- Written equipment-replacement and disposal guidance.
- Condemnation record for utility and engineering equipment.
- Systematic disposal log.

14. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Accreditation Standards for Hospitals, 6th Edition (January 2025) — Facility Management and Safety, standard FMS.4.
- NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition — FMS.4 interpretations.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

This document is not issued by, endorsed by, or affiliated with NABH, the World Health Organization, the National Centre for Disease Control, the Food Safety and Standards Authority of India, any Pollution Control Board, or any other body named in it. Wording is original; no text has been reproduced from the standards, rules or guidelines referenced.